GQUENCE

Clinical Nutrition Platform — Hospital Briefing

What the tool does, who controls it, and what it changes for your department.

Prepared for clinical review

What we are asking you to approve

Stated plainly, before any feature discussion

1

A defined patient group

Adult oncology patients under your care who require nutritional support, enrolled with your consent — not a blanket deployment.

2

A time-boxed evaluation

An agreed review period, after which you decide whether it continues. No lock-in and no long-term commitment.

3

Your existing prescribing authority

Nothing changes about who decides. The tool proposes; you approve. No product reaches a patient without your signature.

What we are NOT asking

- x To change your treatment protocols
- x To replace your dietetics team
- x To integrate with hospital IT systems
- x For any upfront cost during evaluation
- x For patient data to leave your control

What the tool does

Three jobs, in the order they happen

Rx

Calculates

An individualised nutritional prescription from the patient's anthropometry, biochemistry, cancer type and active chemotherapy regimen.

Mf

Produces

The approved prescription becomes a batch-tracked product, made to that specification, labelled and dispatched with full traceability.

Tr

Follows up

Weekly review recalculates the prescription; daily logs record what the patient actually consumed.

The clinical decision stays with you at every step. The tool does the arithmetic, the record-keeping and the follow-up.

Nothing reaches a patient without your approval

Four gates, two of them yours

1

Initial prescription

DOCTOR

The engine proposes energy, protein and formulation. You review, adjust any value, and approve or reject.

2

Production release

STORE APPROVER

Manufacturing cannot begin until a second, independent store role approves the request.

3

Dispatch release

STORE APPROVER

The finished batch cannot leave the store without a separate dispatch approval.

4

Every weekly change

DOCTOR

Each recalculated weekly prescription returns to you as PENDING REVIEW. It does not proceed until you approve it.

A rejected step reverts to the previous state — it is never silently overridden.

It should take work off your team, not add it

What changes on a normal clinic day

Today

- Nutrition assessment done by hand, if time allows
- Calculations repeated per patient, per visit
- No systematic review between appointments
- Deterioration noticed at the next visit
- No record of what the patient actually consumed

With the tool

+ Prescription calculated in seconds, ready for review
+ Weekly recalculation happens automatically
+ Patients needing attention are surfaced for you
+ Deterioration flagged between appointments
+ Consumption recorded day by day

Your added task is review and approval — the clinical judgement only you can provide.

It checks the things that get missed

Drug and nutrient interactions raised at the point of prescribing

16	**19**	**12**	**136**
chemotherapy & radiation interaction rules	automated safety alerts	mandatory investigation triggers	clinical parameters you can inspect and change

Example — a patient on Cisplatin

- **Renal magnesium wasting** — magnesium protocol raised automatically
- **Escalation trigger** — creatinine > 1.5 mg/dL — hold and escalate
- **Nephrotoxicity** — weekly creatinine monitoring enforced
- **Protein adequacy** — minimum threshold raised for catabolism

Every rule is visible and editable — you can see exactly why the tool said what it said.

It watches your patients between appointments

Ranked against ESPEN / GLIM malnutrition criteria

Patients Needing Attention

5 Critical | 6 High | 4 Supply | 3 Not monitored

- **Rapid weight loss** — 3.8%/week — exceeds the 2%/week malnutrition threshold
- **Inflammatory catabolism** — albumin falling while CRP rises

Reasoning always shown

Each flag states the measurement, the threshold crossed, and why it matters. Never a bare score.

Tuned against real data

Thresholds were tightened using this cohort so the list stays short enough to act on.

You decide what to do

It surfaces patients. It does not change treatment or contact anyone.

Every action is attributed and timestamped

What the record shows if anyone ever asks

Recorded for every product

Who calculated it	user, role and timestamp
Who approved it	doctor identity and time of approval
What was approved	the exact specification, not a summary
Who produced it	store operator and batch number
Who released it	independent approver at each stage
When it was dispatched	and to which patient

Access control

Six roles, each restricted to what that role needs:

Doctor	own patients, full clinical record
Assistant	data entry under a named doctor
Store Manager	production queue only
Store Approver	release authority only
Admin	configuration and reporting
Super Admin	user and site management

Store staff never see clinical notes. Clinical staff never touch batch release.

Patient data

Where it sits and who can reach it

Stored

Encrypted database, access restricted by role. No patient record is visible to a user whose role does not require it.

Identified

Patients carry a hospital UHID and a study ID. Exports for research use the study ID, not the name.

Shared

Nothing is sold, shared with third parties, or used for advertising. The data belongs to the hospital.

Removed

On request, a patient can be withdrawn and their record excluded from all reporting and exports.

We will follow whatever data agreement your institution requires, including keeping data within your infrastructure.

What the department gets

Beyond the individual patient

01	**A defensible nutrition pathway**	A documented, consistent standard applied to every patient — replacing ad hoc calculation that varies by who is on duty.
02	**Visibility between visits**	Deterioration surfaced when it happens rather than at the next appointment, when the window to act has narrowed.
03	**Research-grade data at no extra effort**	Longitudinal nutritional data accrues from routine care. Exportable, analysis-ready, and available for your own publications.
04	**A record that stands up to scrutiny**	Full chain of custody per patient, which is what audit and accreditation ask for and paper systems rarely provide.

Co-authorship on any publication arising from data generated at your site.

What if something goes wrong

The questions you should be asking

If the calculation is wrong	You see the full specification before approving, and can change any value. The tool cannot act on an unapproved plan.
If the system is unavailable	Prescribing reverts to your existing process. No patient depends on the software being up.
If a patient reacts badly	Batch number, exact composition and dispatch date are on record for every product, so any reaction is traceable to a specific batch.
If you want to stop	Withdraw a patient or end the evaluation at any point. Your data is exported to you and the deployment stops.

Proposed next step

1

Review the engine

Sit with us for an hour and check the clinical logic against your own protocols. Tell us where it is wrong.

2

Agree the patient group

You define who is eligible and how many. We start small.

3

Set a review date

An agreed point at which you decide whether it continues, stops or expands.

Already in place

- Live in a production clinic
- Full audit trail per patient
- Six governed user roles
- Clinical source documentation
- Research export built in